

CSV

Choose File

No file chosen

Core

subject_id

session

diagnosis

behavior_label

▼ Label guide

Undiagnosed

No study diagnosis — comparison group

Diagnosed

Meets study criteria for the target condition

Stable

Behavior within expected range for group

Watchlist

Mild behavioral shift — monitor across sessions

Elevated

Marked behavioral change — review clinically

Sensors

☒ EEG alpha power

☒ EEG theta/beta ratio

☒ HRV (RMSSD)

☒ Sleep efficiency (%)

☒ Gait variability index

Threshold

0.5

C

1.0

▼

Impute

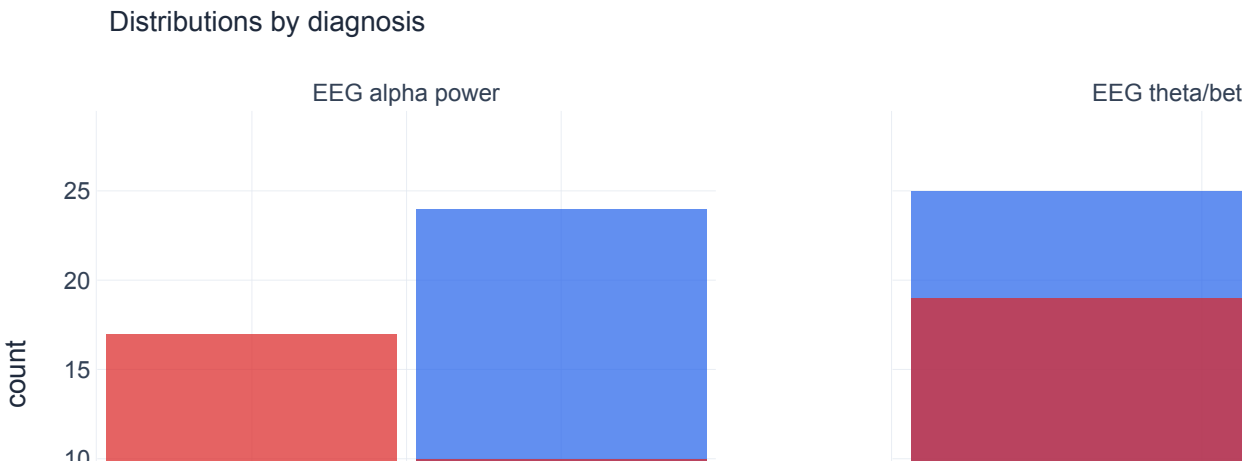
mean

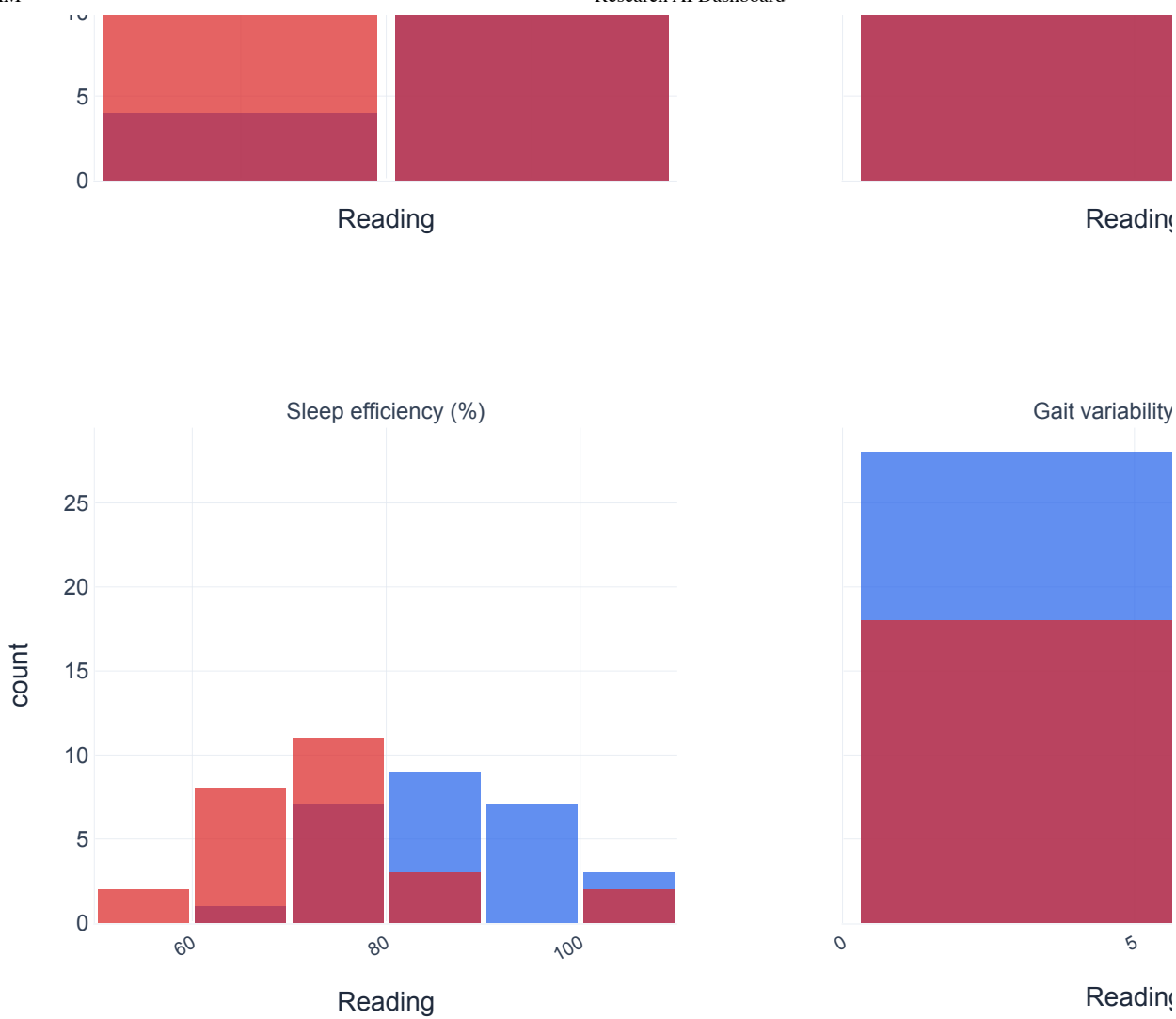
▼

A summary view showing how many visits and readings were used, split by diagnosis and behavior. The chart is meant to give a quick picture of the whole group of people and their repeated visits. It shows that diagnosed and undiagnosed groups are represented, so differences can be seen across time. It also highlights that behavior levels are part of the picture, not just diagnosis labels. Overall, it suggests the study is tracking real-world variation, including people who may look different from one visit to the next.

Data Visualization

Explore sensor readings, missing data, and how participants group by diagnosis and visit.



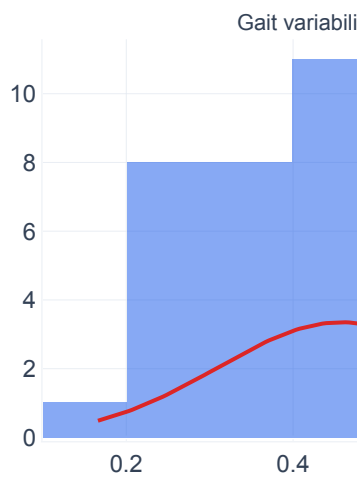
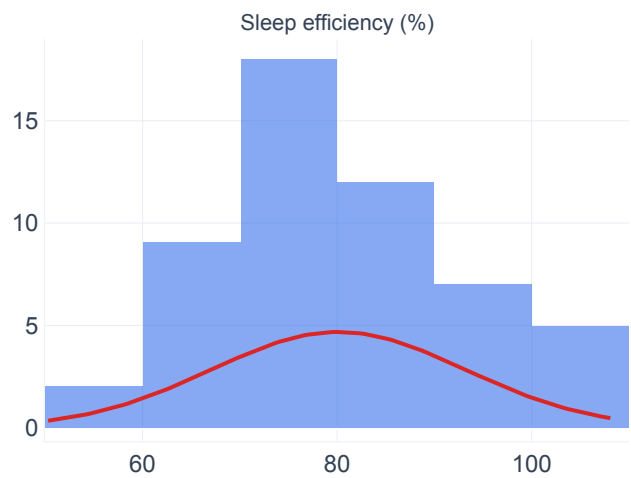
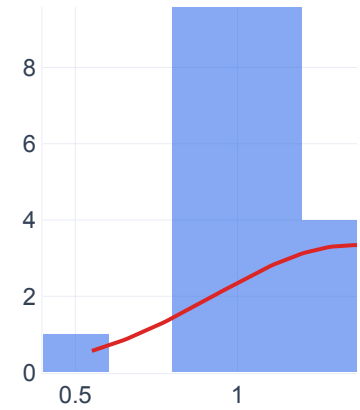
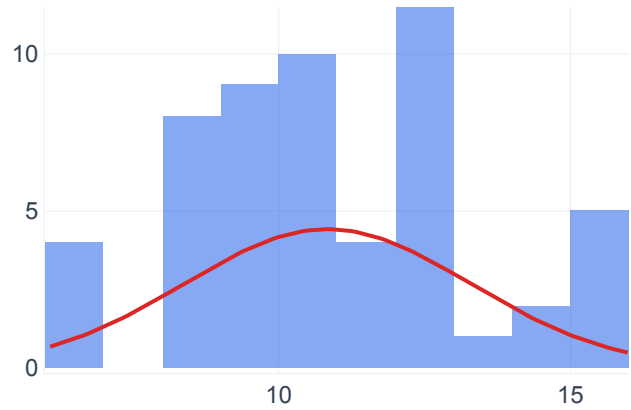


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Two overlapping histograms showing typical sensor levels for diagnosed versus undiagnosed groups. If the two shapes are separated, it suggests diagnosed people tend to show consistently different levels. If they overlap heavily, it means many individuals share similar sensor ranges, making classification harder. For patients, overlap often corresponds to mild symptoms or day-to-day changes, where the body signals do not perfectly match the diagnosis label.

Sensor distributions with normal-curve reference



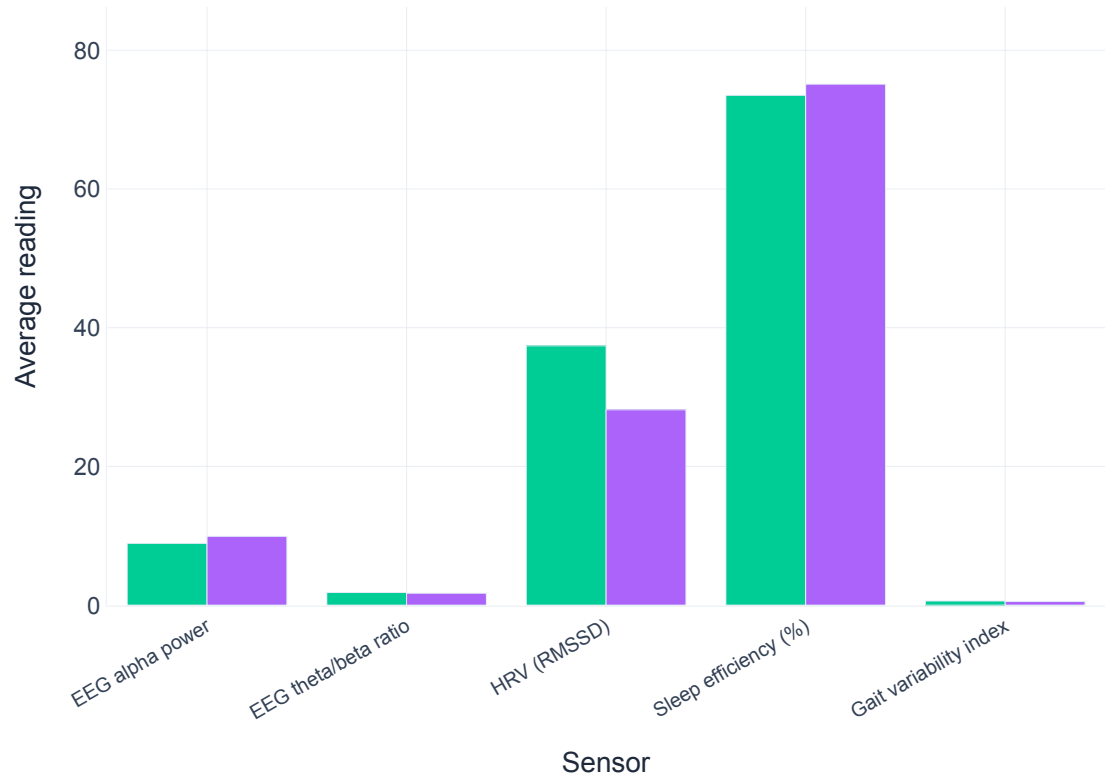


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Smoothed, bell-shaped curves for each group showing where most readings fall. These curves summarize the spread of readings within each group. When the peaks are far apart, it suggests diagnosed and undiagnosed people have distinct typical patterns. When the curves overlap, it indicates that individual readings can look similar across diagnoses. For patients, this means some people may sit near the middle of the range, where symptoms and physiology can be harder to distinguish.

Average sensor levels: first vs second visit by diagnosis

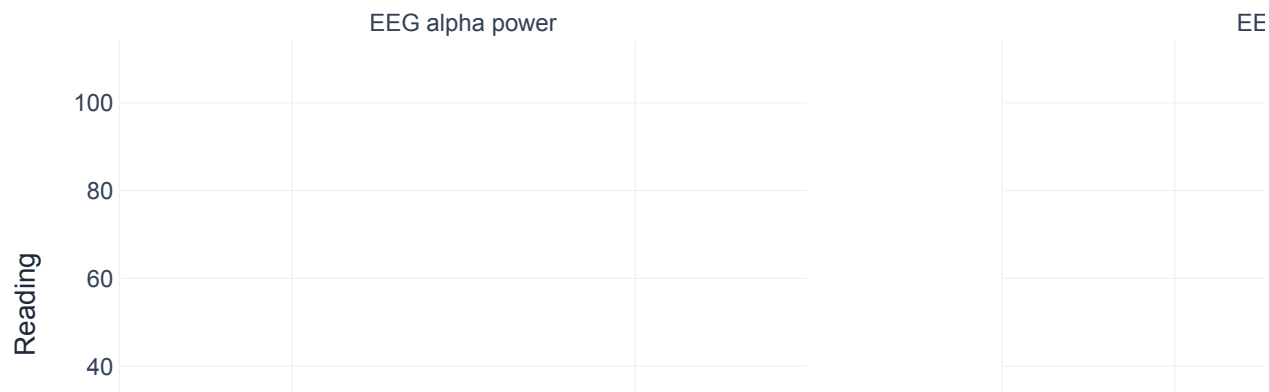
Diagnosed

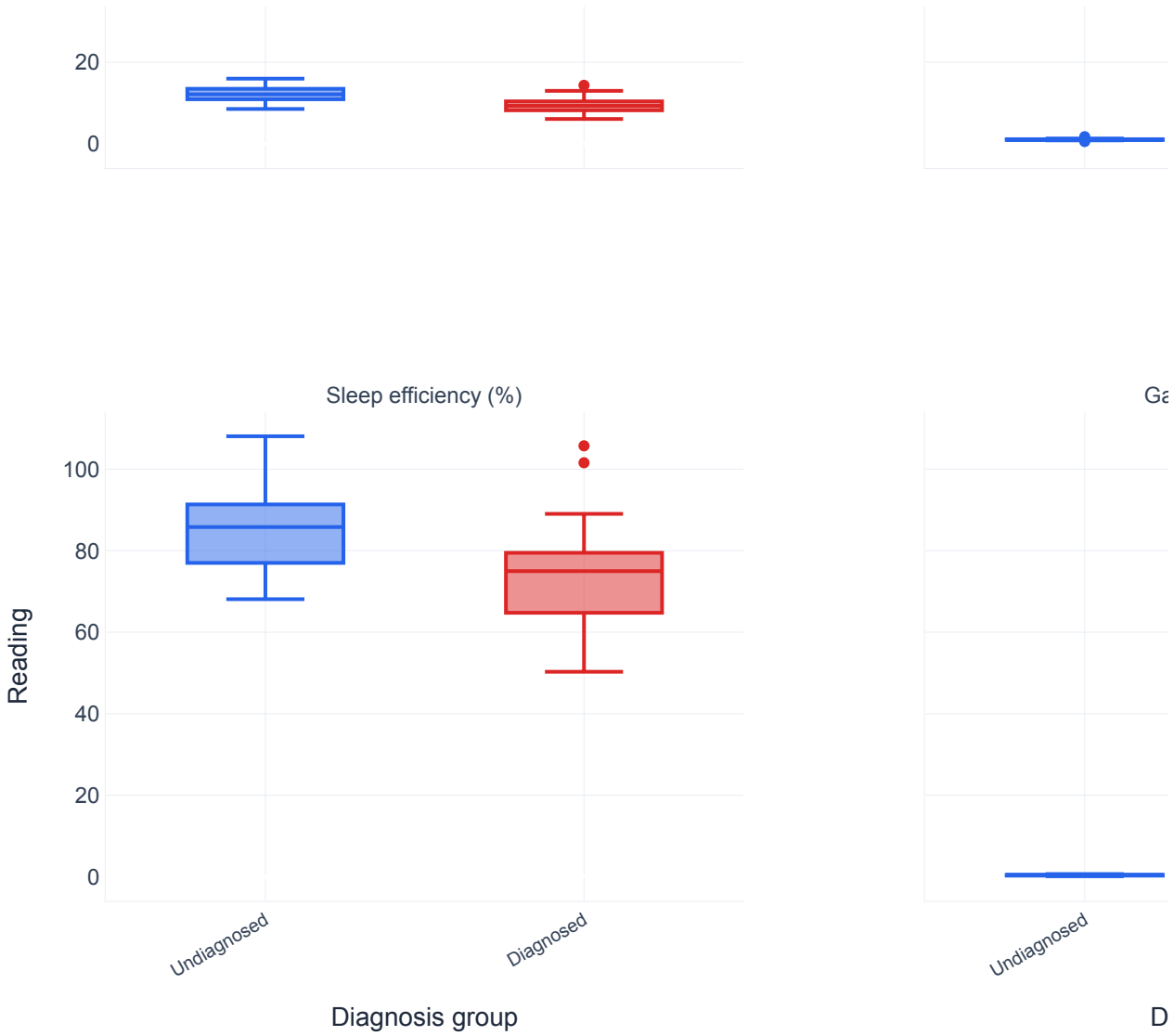


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A side-by-side comparison of average sensor levels on visit one versus visit two, split by diagnosis. This chart shows whether sensor levels change from the first to the second visit, and whether that change differs by diagnosis. If diagnosed and undiagnosed lines move differently, it suggests the body signals track with diagnosis over time. If both groups move similarly, it suggests changes may be driven by general factors like sleep or recovery rather than diagnosis alone. For patients, it reflects how stable or variable symptoms and physiology can be across visits.

Sensor spread by diagnosis group

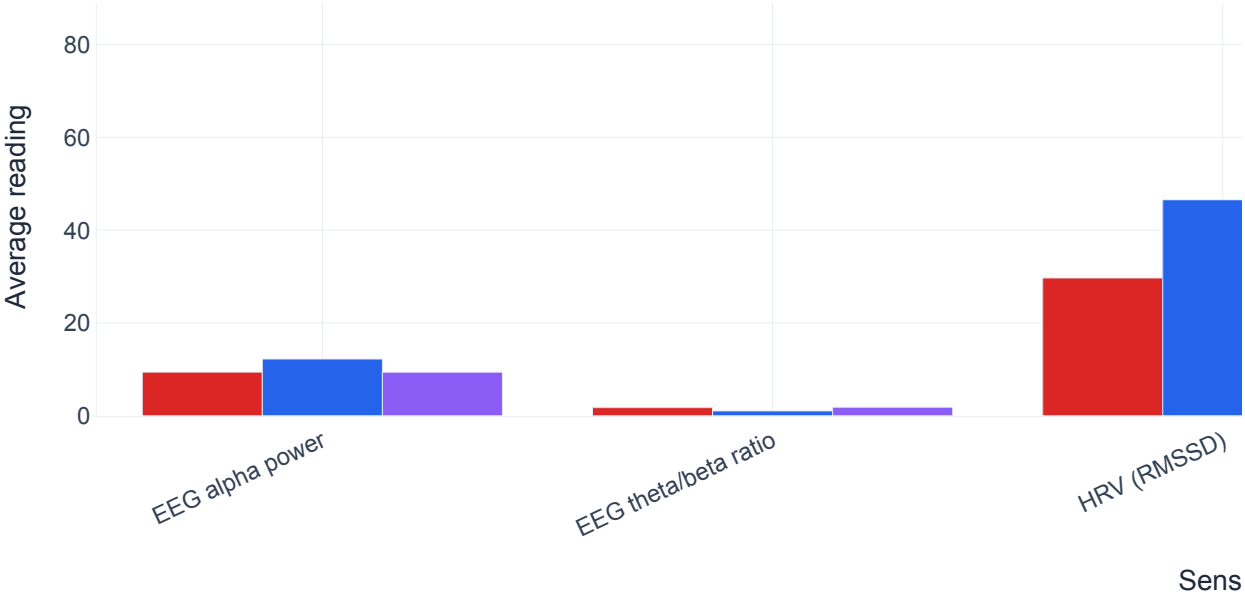




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Box-and-whisker plots showing the spread of each sensor within diagnosed and undiagnosed groups. Each box shows the typical range of sensor values and how much they vary within a group. If diagnosed boxes are shifted higher or lower than undiagnosed, it suggests a consistent difference in that signal. If boxes overlap widely, it means individual patients can look similar despite different diagnosis labels. For patients, wide spread indicates variability, which can happen when symptoms fluctuate or when readings are affected by comfort, movement, or sleep.

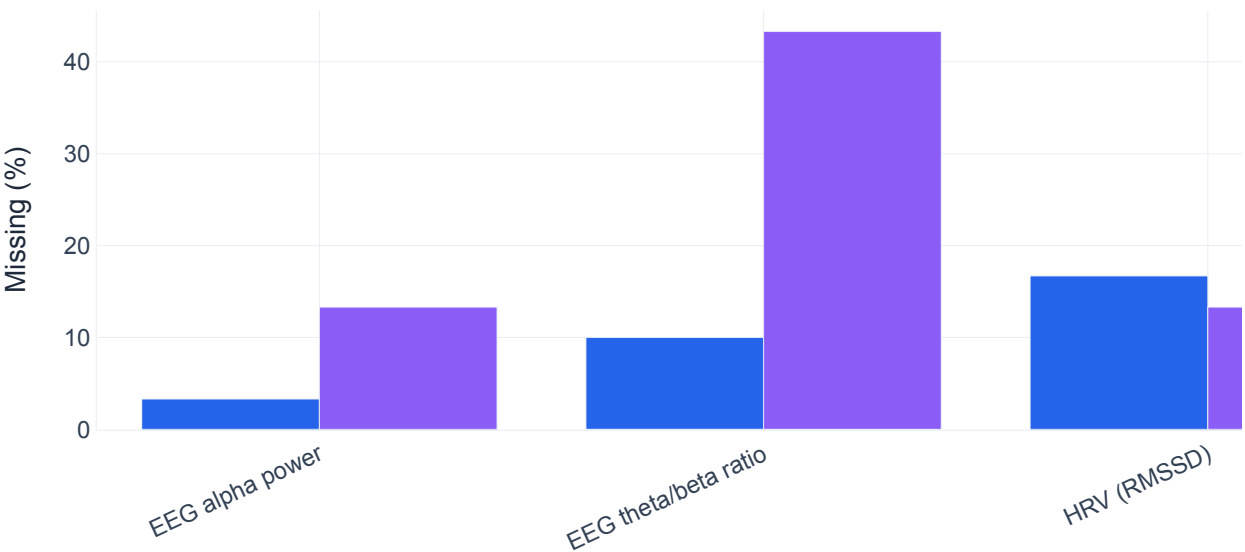
Average sensor levels by behavioral status



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A chart showing how average sensor levels change across behavior tiers. This chart links behavior level with body signals, showing whether more severe behavior is associated with stronger or weaker sensor patterns. If sensor averages rise or fall steadily across tiers, it suggests a meaningful relationship between day-to-day behavior and physiology. If the pattern is inconsistent, it suggests behavior may not map neatly onto the signals for every person. For patients, it reflects that behavior and symptoms can move together for some, but not always for others.

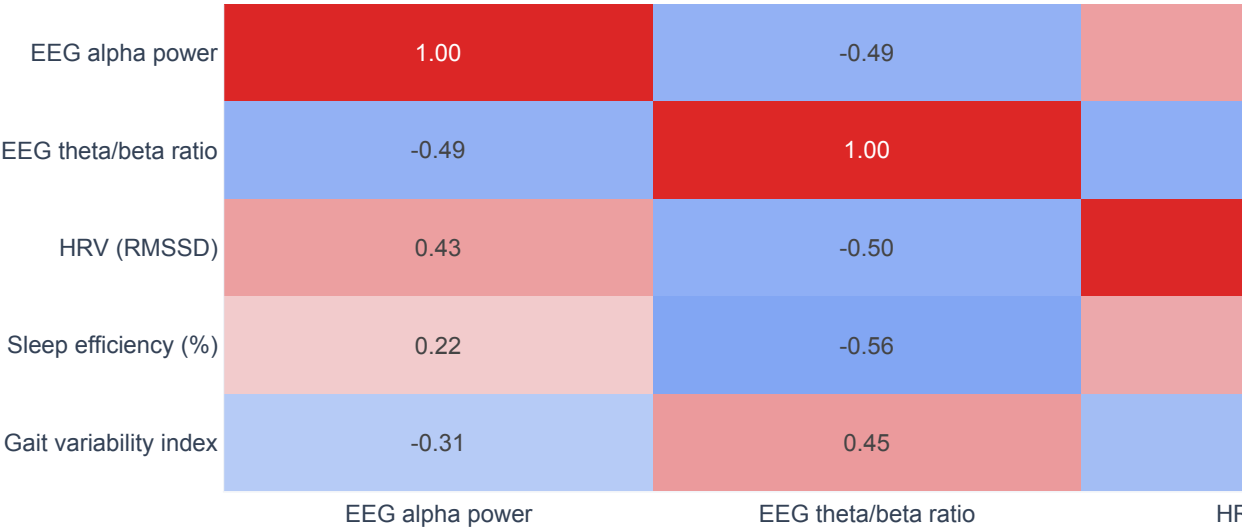
Missing readings by sensor and visit



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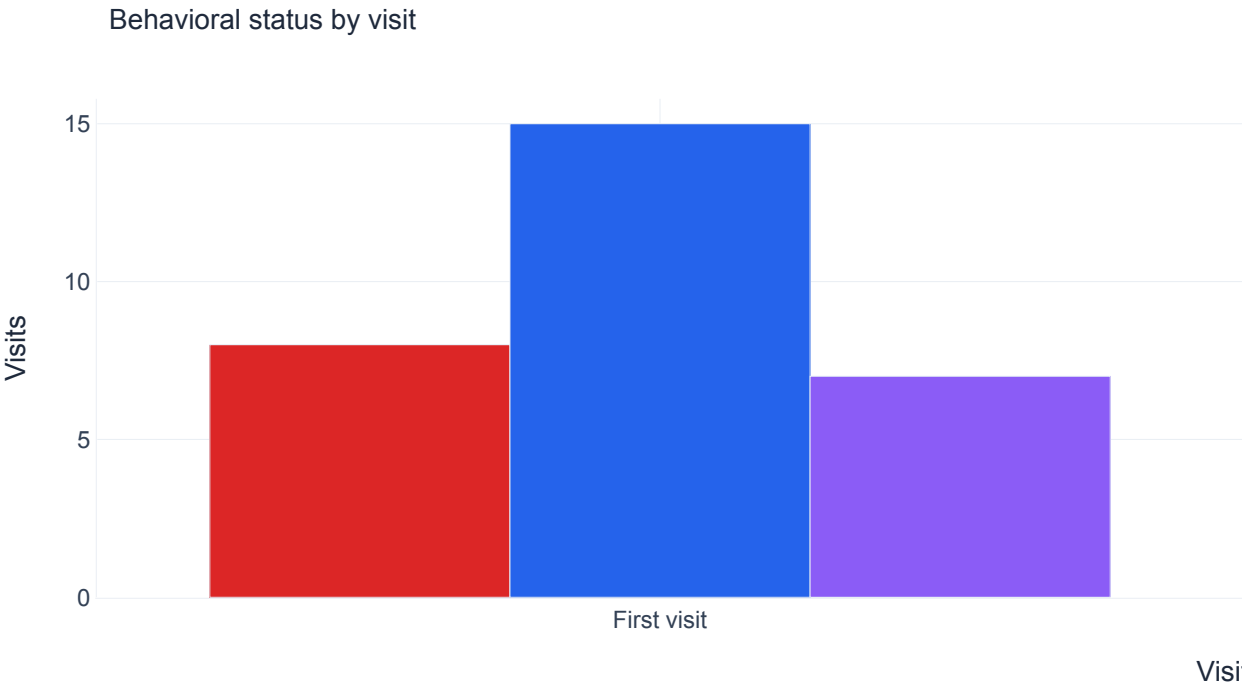
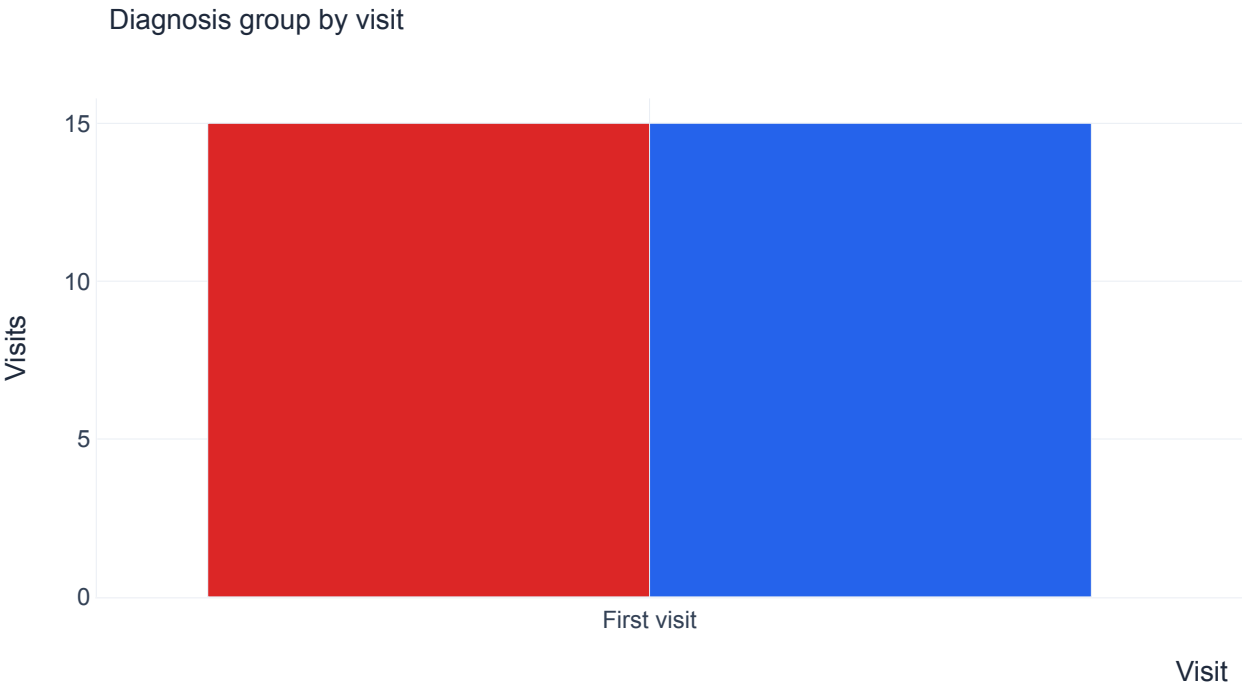
A bar chart comparing how many readings are missing on the first versus the second visit. This chart shows whether data collection was more complete on one visit than the other. If more readings are missing on the second visit, it suggests some patients had more difficulty completing the measurements later. If missingness is similar across visits, it suggests the measurement process was stable. For patients, missing readings can hide real struggle, especially if the missingness is more common in one diagnosis group.

Sensor correlation matrix



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A grid of colored squares showing which sensors move together and which move independently. This chart highlights pairs of signals that tend to rise and fall together. For patients, coordinated signals may reflect a consistent physiological state, while independent signals may reflect mixed or fluctuating symptoms.



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A breakdown chart showing how many visits fall into each diagnosis and behavior tier. This chart shows how the visits are distributed across diagnosis and behavior levels. Balanced counts mean the study includes a fair mix of typical and more challenging presentations. If one group is much larger, it can make patterns look stronger simply

because there are more examples. For patients, it helps explain whether the findings reflect many kinds of symptom severity or mostly one end of the spectrum.

Field	Missing (%)
EEG theta/beta ratio	26.7
Gait variability index	23.3
HRV (RMSSD)	15.0
Sleep efficiency (%)	11.7
EEG alpha power	8.3
Participant ID	0.0
Visit	0.0
Diagnosis	0.0
Behavior status	0.0

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A table listing which readings are most often absent, with darker shading for more missing values. This chart identifies which sensors are hardest to capture reliably. When certain readings are missing often, it means some patients’ experiences may be underrepresented in those signals. If missingness is higher in diagnosed people, it can hide differences that would otherwise be visible. For patients, missing readings can make the overall picture look less clear, even when symptoms are present.

Model Results

How the support model compares to recorded diagnoses on this dataset.

Metric	Value
Accuracy	0.783
Precision	0.698
Recall	1.000
F1 Score	0.822
ROC AUC	0.954

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A set of bars summarizing how often the computer’s diagnosis matches the chart diagnosis. This chart tells how consistently the computer agrees with what clinicians recorded. When agreement is high, it means the readings often line up with the diagnosis label. When agreement is lower, it means some visits are harder to classify, even though the chart diagnosis is known. For patients, this reflects that symptoms and physiology can overlap, especially when symptoms are mild or fluctuate between visits.

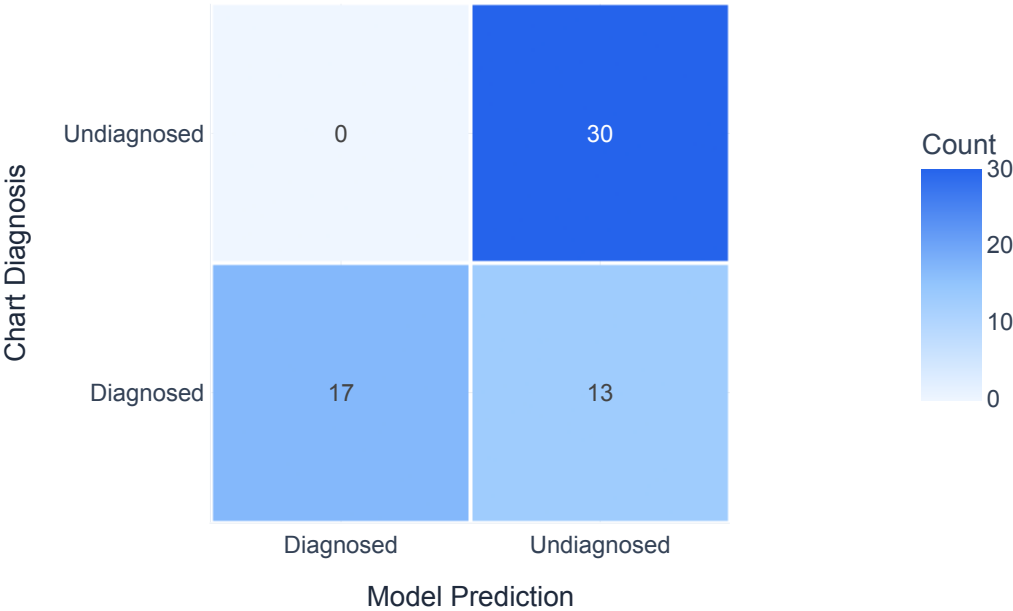
Current cutoff is near 0.50 (78.3% agreement on this dataset).

Confidence cutoff	Agreement with chart (%)	Diagnosed cases caught (%)
0.35	78.3	56.7
0.40	78.3	56.7
0.45	78.3	56.7
0.50	78.3	56.7
0.55	80.0	60.0
0.60	83.3	66.7
0.65	85.0	70.0

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A line chart showing how the number of positive results changes as the confidence cutoff moves. This chart shows how results shift when the computer requires more or less confidence before calling a visit diagnosed. When the curve changes sharply, small changes in confidence can flip many visits, reflecting borderline cases. When the curve changes slowly, the system’s calls are more stable. For patients, this means some visits sit near the dividing line, where symptoms and physiology overlap and the label can vary depending on strictness.

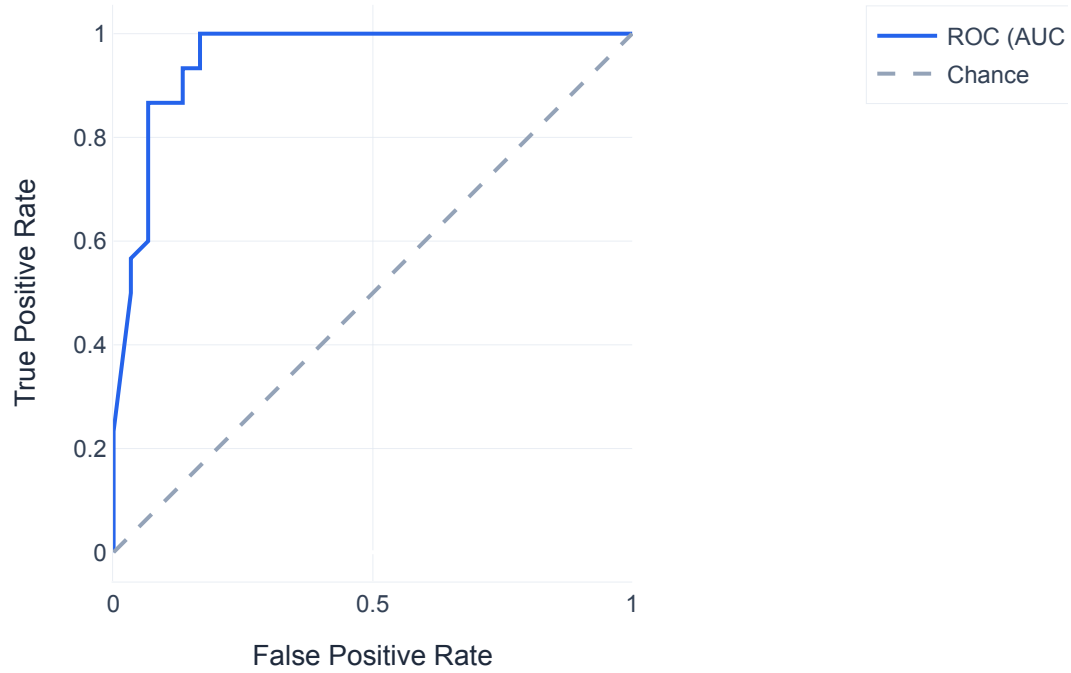
Confusion Matrix



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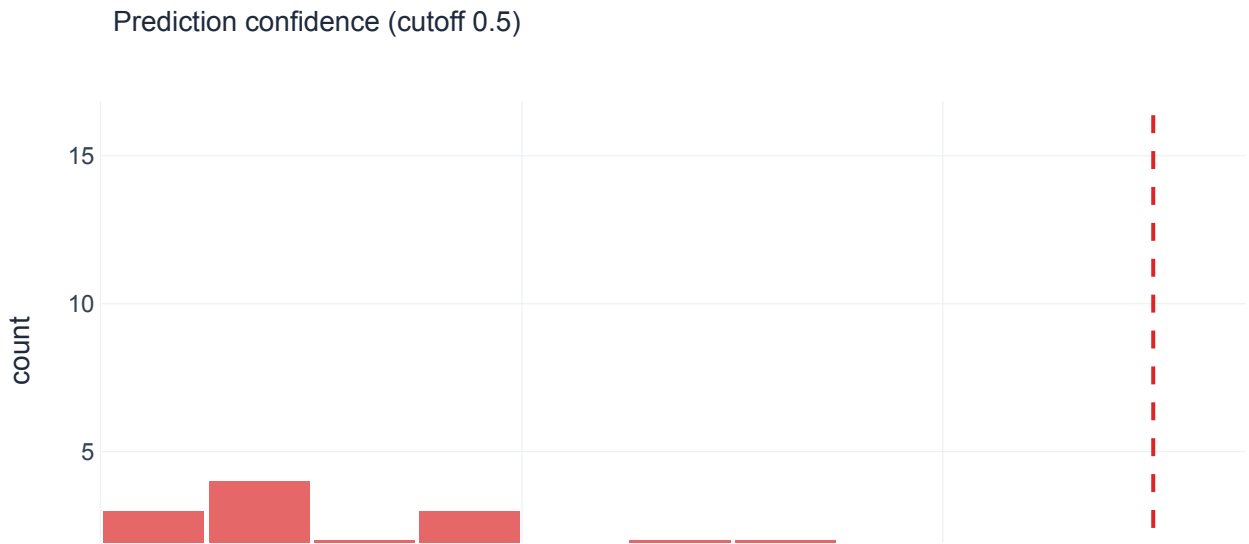
A square grid showing correct matches on the diagonal and mix-ups off the diagonal. The diagonal cells represent visits where the computer and the chart diagnosis match. This helps explain which group is more likely to be confused. For patients, it suggests that some people—often those with milder or changing symptoms—may have readings that resemble the other group.

ROC Curve



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A curve rising above the diagonal line, showing trade-offs between catching diagnosed and avoiding false alarms. This chart shows how well the readings separate diagnosed from undiagnosed across different confidence levels. A curve that stays well above the middle line suggests clearer separation, meaning the two groups tend to have different patterns. If the curve is closer to the middle, separation is weaker and overlap is more common. For patients, it means some visits will naturally be ambiguous, especially when symptoms are mild or readings are affected by day-to-day factors.

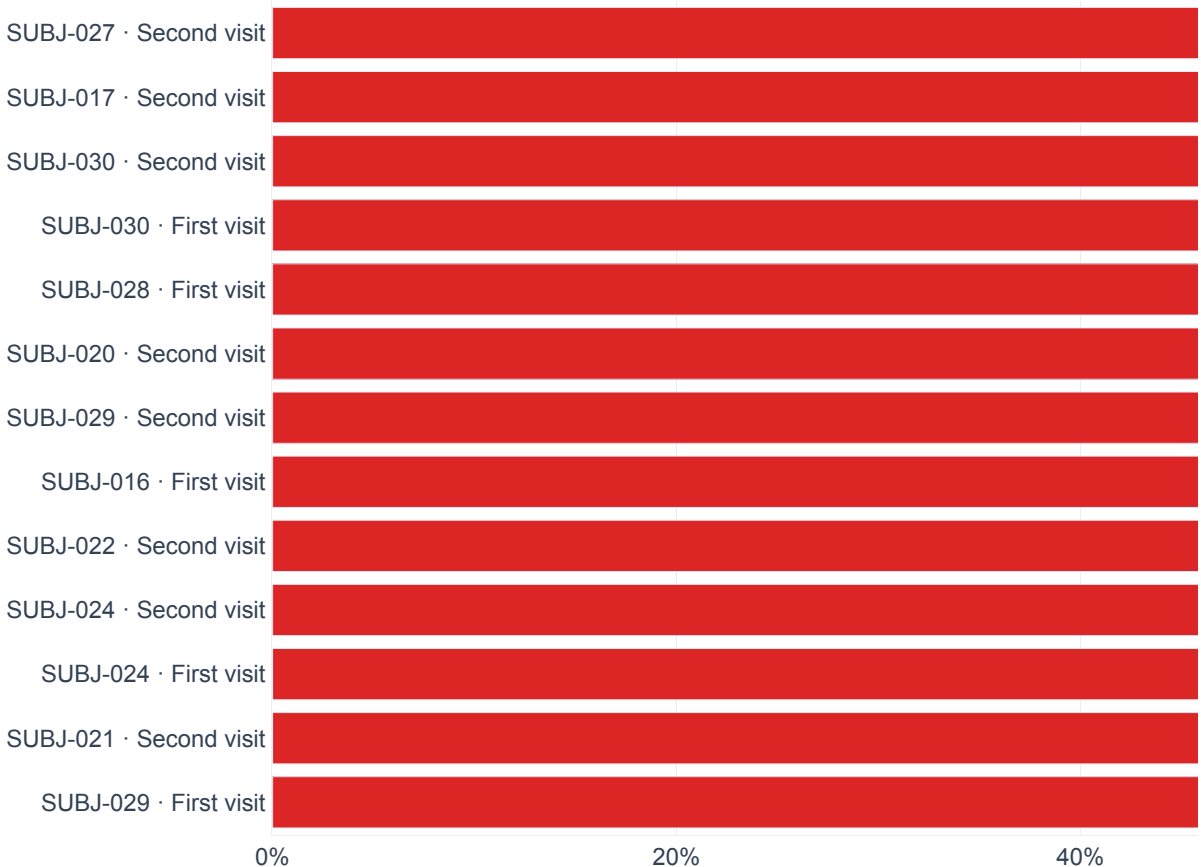




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A distribution of confidence scores across visits, with many values clustered and some spread out. This chart shows how confident the computer is for each visit, not just the final label. When many visits have high confidence, it suggests the readings often give a clear signal. When confidence is spread out or frequently moderate, it suggests uncertainty and overlap between groups. For patients, this reflects that some visits look more typical of the diagnosis, while others look mixed, which can happen when symptoms vary over time.

Visits where the model disagreed with the chart diagnosis



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A list-like summary highlighting visits where the computer's label disagrees with the chart diagnosis. This chart focuses on the "disagreement" visits, which are the hardest cases. It suggests that some people may have readings that shift toward the other group, even though the chart diagnosis stays the same. Disagreements can be more common when symptoms are mild, when physiology is influenced by sleep or stress, or when readings are incomplete. For patients, it means the system struggles most when the picture is not clear-cut.